

A FOUNDRY RX GUIDE

The Microdosing *Playbook.*

Low-dose GLP-1s, explained. What the research says, what the hype gets wrong, and how to start smart.



WELCOME

Weight loss is being *rethought.*

For years, the GLP-1 playbook was simple: climb to the highest dose you can tolerate. A growing number of clinicians are asking a different question. What is the least you actually need?

That question is what microdosing is about. This guide walks through the ideas behind it, the research that exists (and where it runs out), how to protect your muscle, and what to ask before you start with any provider, including us.

No hype, no miracle claims. Just what a smart, informed patient should know.

INSIDE

01	Why lower doses are the new conversation	3
02	Appetite control, not appetite loss	4
03	Lose weight, keep your muscle	5
04	GLP-1s after 40	7
05	Side effects, and why starting low matters	8
06	Compounded vs. branded, plainly explained	9
07	Hot topics: what's emerging	10
08	10 questions to ask any GLP-1 provider	11



CHAPTER 01

Why lower doses are *the new conversation.*

Microdosing moved from niche to mainstream through long-form health podcasts. A widely shared episode of *Boundless Life* featured Dr. Tyna Moore, a naturopathic physician, making the case for three ideas: use lower doses, dose for appetite control rather than suppression, and protect muscle along the way.

The appeal is intuitive. If a smaller dose quiets cravings, why take more?

THE HONEST PART

The large GLP-1 trials tested standard dose escalations, not microdosing protocols. Microdosing today is a clinician-guided approach built on those trials plus individual response. Anyone who promises specific microdose results is getting ahead of the evidence.

Foundry RX is not affiliated with or endorsed by Boundless Life, Ben Greenfield, or Dr. Tyna Moore.

Appetite control, *not appetite loss.*

Most people don't struggle with hunger. They struggle with food noise: the constant background pull toward snacks, second helpings, and late-night kitchen visits.

What GLP-1s do

GLP-1 medications mimic a hormone your gut releases after eating. They slow how quickly your stomach empties and act on appetite centers in the brain, so you feel full sooner and stay full longer.¹

Where high doses overshoot

At higher doses, some people describe appetite shutting off almost completely. That can make it hard to eat enough protein, enjoy meals, or keep up training.

The microdose goal

Turn the noise down, not off. You still get hungry. You still enjoy a good meal. You just stop thinking about food all day.

Quiet the noise.
Keep the pleasure of a
good meal.

Everyone responds differently. Your provider adjusts your dose based on how you feel, not a fixed schedule.

Lose weight, *keep your muscle.*

Any meaningful weight loss includes some lean mass, not just fat. The question is how much, and what you can do about it.

~40%

Up to about 40% of weight lost was lean mass in body-composition data from the major semaglutide trials. Across GLP-1 studies, the share varies widely, from about 15% to more than 40%.²

1.2–1.6_{g/kg}

Daily protein range recommended during active weight loss in a 2025 joint advisory from four leading obesity and nutrition organizations. The same advisory calls for strength training alongside GLP-1 therapy.³

2–3_{x/wk}

Resistance training is the most consistent recommendation for preserving muscle while losing weight. It doesn't need to be heavy. It needs to be regular.

The good news: in the same trial data, the proportion of the body that was lean actually improved, because fat dropped faster than muscle.² The goal is to push that ratio further in your favor.

Protein is your *muscle insurance*.

A simple starting point: 1.2 to 1.6 grams of protein per kilogram of body weight per day.³ Here's what that looks like in pounds.

YOUR WEIGHT	DAILY PROTEIN	PER MEAL (3 MEALS)
140 lbs	76–102 g	25–34 g
170 lbs	93–123 g	31–41 g
200 lbs	109–145 g	36–48 g
230 lbs	125–167 g	42–56 g
260 lbs	142–189 g	47–63 g

Your provider may personalize this, especially if you have kidney disease or other conditions.

Make it easy

- ✓ **Protein first.** Eat it before the rest of the plate, while appetite is highest.
- ✓ **Spread it out.** 30–40 g per meal beats one big dinner.
- ✓ **Keep backups.** Greek yogurt, eggs, cottage cheese, a clean shake.

Train to keep it

- ✓ **2–3 strength sessions** a week, full body.
- ✓ **Progress slowly.** A little more weight or a few more reps over time.
- ✓ **Walk daily.** It supports everything else.

GLP-1s *after 40.*

The body you're working with at 45 isn't the one you had at 25.
The approach should change with it.

Muscle gets harder to keep

Muscle mass naturally declines with age, and it's harder to rebuild once lost. That makes protecting it during weight loss more important, not less. It's a big part of why the lowest-effective-dose idea resonates with people in midlife.

Hormones shift the picture

For many women, perimenopause and menopause bring changes in where the body stores fat and how appetite behaves. For many men, gradual hormonal changes affect energy and body composition. Share the full picture with your provider.

Other medications matter

Midlife often means more prescriptions. GLP-1s slow stomach emptying, which can affect how some oral medications are absorbed. Your provider should review everything you take.

MIDLIFE CHECKLIST

- ✓ Share every medication and supplement
- ✓ Prioritize protein and strength training
- ✓ Track how you feel, not just the scale

Side effects, and why *starting low matters*.

The most common GLP-1 side effects are digestive: nausea, vomiting, diarrhea, constipation, and stomach discomfort. They tend to show up when doses increase.¹

How people manage them

- ✓ Smaller meals, eaten slowly
- ✓ Stop at comfortably full, not stuffed
- ✓ Go easy on greasy, rich foods early on
- ✓ Hydrate throughout the day
- ✓ Tell your provider. Dose timing and size can be adjusted.

Why low and slow

A smaller starting dose with gradual adjustments gives your body time to adapt. For many people, that's the difference between sticking with treatment and quitting in week three.

GET CARE RIGHT AWAY IF YOU HAVE

Severe or persistent stomach pain, pain that spreads to your back, signs of an allergic reaction, or symptoms of dehydration. These can signal serious problems such as pancreatitis or gallbladder issues.

Compounded vs. branded, *plainly explained.*

You'll see both on the market. Here's the difference, with no spin.

	BRANDED	COMPOUNDED
FDA approval	FDA-approved	Not FDA-approved. The FDA does not review compounded drugs for safety or effectiveness.
Made by	The drug manufacturer	A licensed compounding pharmacy or outsourcing facility
Dosing	Fixed pen doses	Can be prepared in smaller, adjustable amounts, which is why it's common in microdosing
Requires	A prescription	A prescription

503A vs. 503B

503B outsourcing facilities register with the FDA, are inspected by it, and follow federal manufacturing standards (cGMP). 503A pharmacies are state-regulated and compound prescriptions for individual patients. Foundry RX medications come from a U.S. 503B facility.

Rules for compounded GLP-1s continue to evolve. A good provider tells you where your medication comes from, how it's made, and what's changing.

Hot topics: *what's emerging.*

Early findings, not proven benefits. We include them because you'll hear about them, and you deserve the real picture.

48

Alcohol cravings. In a 2025 randomized trial of 48 adults with alcohol use disorder, low-dose semaglutide reduced drinks per drinking day and alcohol craving compared with placebo. The authors called it initial evidence that justifies larger trials.⁴

2/3

Stopping and regain. One year after stopping semaglutide in the STEP 1 extension, participants had regained about two-thirds of the weight they'd lost.⁵ It's one reason clinicians increasingly talk about the lowest dose you can sustain, and a real plan for the long term.

5–18%

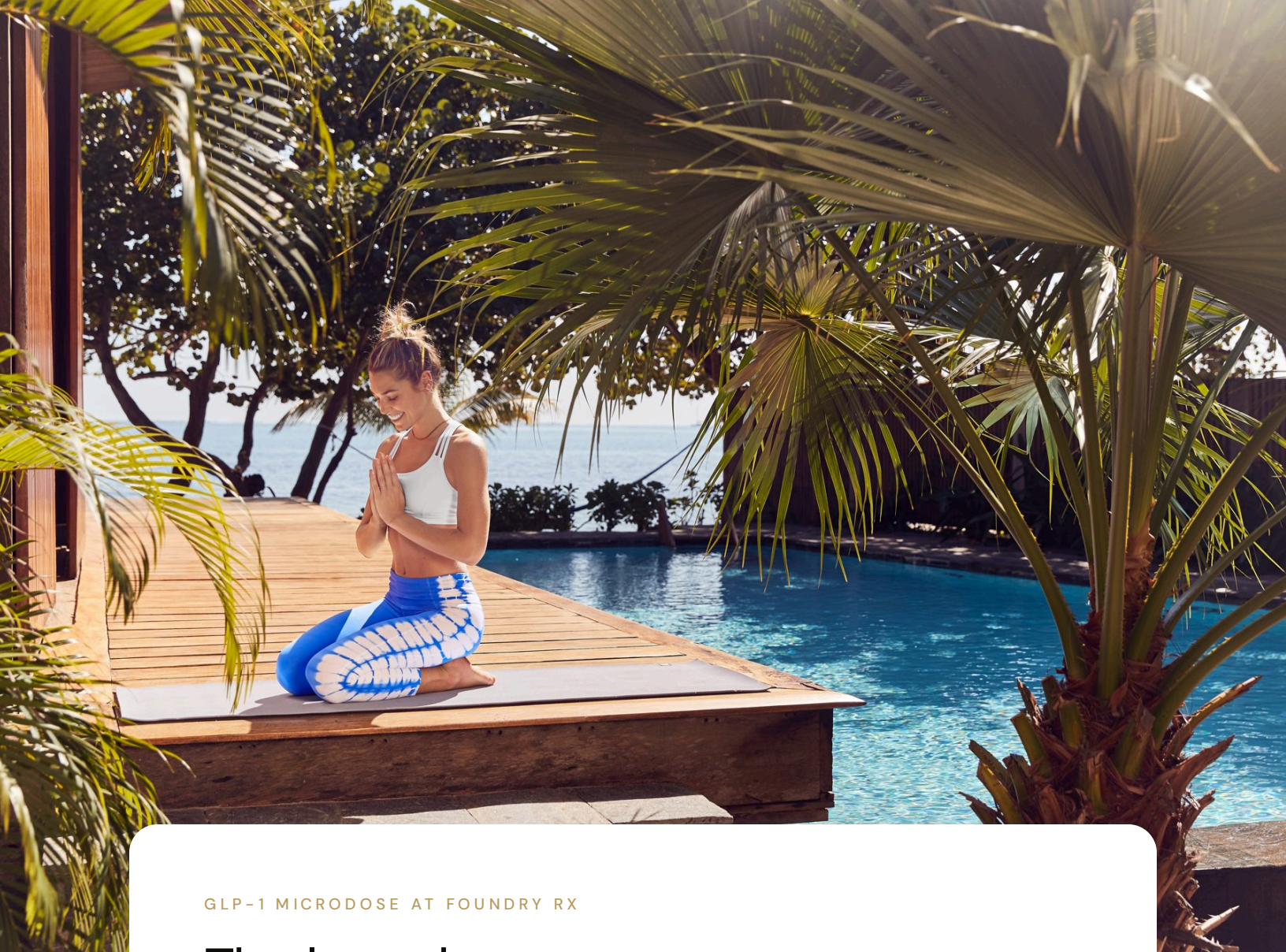
Beyond the scale. GLP-1 therapies have shown average placebo-adjusted weight reductions of 5–18% in trials, along with improvements in metabolic and cardiovascular measures.³ Whether microdosing delivers similar benefits hasn't been studied in large trials.

The takeaway: GLP-1 science is moving fast. Choose a provider who keeps up with it and tells you what's proven versus promising.

10 questions to ask *any GLP-1 provider.*

Bring these to your first visit, wherever you go.

-
- 01 Who reviews my intake, and what are their credentials?
 - 02 How fast will I hear back, and how do I reach my provider later?
 - 03 Where is my medication made? Is it FDA-approved, 503A, or 503B?
 - 04 How will you set and adjust my dose? Fixed schedule or based on my response?
 - 05 What's the plan to protect my muscle?
 - 06 Which side effects should I report, and which need urgent care?
 - 07 Does the price change after month one? What's included?
 - 08 Does the price include the medication, or is that extra?
 - 09 What happens if I want to stop? Is there a plan for maintenance?
 - 10 Will you review all my other medications and conditions first?



GLP-1 MICRODOSE AT FOUNDRY RX

The low-dose *way to start.*

\$99 /month*

Same price all 3 months. No teaser pricing.

[See if you qualify](#)

foundryrx.co/glp1-microdose

- ✓ 24-hour provider review
- ✓ Consultation included
- ✓ Dose fine-tuned as you go
- ✓ U.S. 503B-compounded

*\$99/month with a 3-month plan, billed \$297 every 3 months. \$119/month month-to-month. Prescription only. A licensed provider decides if treatment is right for you.

SOURCES

1. U.S. Food and Drug Administration. Prescribing information for semaglutide (Wegovy) and related GLP-1 receptor agonists: mechanism of action and adverse reactions.
2. Neeland IJ, Linge J, Birkenfeld AL. Changes in lean body mass with glucagon-like peptide-1-based therapies and mitigation strategies. *Diabetes Obes Metab*. 2024. doi:10.1111/dom.15728
3. Mozaffarian D, et al. Nutritional priorities to support GLP-1 therapy for obesity: a joint Advisory from the American College of Lifestyle Medicine, the American Society for Nutrition, the Obesity Medicine Association, and The Obesity Society. *Am J Clin Nutr*. 2025;122(1):344–367. doi:10.1016/j.ajcnut.2025.04.023
4. Hendershot CS, et al. Once-weekly semaglutide in adults with alcohol use disorder: a randomized clinical trial. *JAMA Psychiatry*. 2025;82(4):395–405. doi:10.1001/jamapsychiatry.2024.4789
5. Wilding JPH, et al. Weight regain and cardiometabolic effects after withdrawal of semaglutide: the STEP 1 trial extension. *Diabetes Obes Metab*. 2022;24(8):1553–1564. doi:10.1111/dom.14725

IMPORTANT SAFETY INFORMATION

GLP-1 medications are available by prescription only after evaluation by a licensed provider. Do not use if you or a family member have a history of medullary thyroid carcinoma (MTC) or Multiple Endocrine Neoplasia syndrome type 2 (MEN2), if you have had a serious allergic reaction to a GLP-1 medication, or if you are pregnant, breastfeeding, or planning a pregnancy. GLP-1 medications may cause serious side effects, including possible thyroid tumors, pancreatitis, gallbladder problems, low blood sugar, kidney problems, and changes in mood. Common side effects include nausea, vomiting, diarrhea, constipation, and abdominal pain. Tell your provider about all medications you take. Seek care right away for severe stomach pain or signs of an allergic reaction.

Compounded medications are prepared by a licensed 503B outsourcing facility and are not FDA-approved. The FDA does not verify the safety or effectiveness of compounded drugs. Individual results vary.

This guide is for general education and is not medical advice. It does not replace a consultation with a licensed healthcare provider. Microdosing refers to clinician-guided use of lower-than-typical doses and has not been evaluated in large clinical trials.

Foundry RX is a DBA of Algorithm Management Inc. Care is provided by independent licensed clinicians. Foundry RX is not affiliated with or endorsed by Boundless Life, Ben Greenfield, or Dr. Tyna Moore. © 2026 Foundry RX. members@foundryrx.co